



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : مني احمد طه خليفه
المحافظة : القليوبيه
الحى / المدينة : الخانكة
رقم البطاقة : 26010241400666 رقم التليفون : 01004775454
العنوان : الخانكة
العمر (تاريخ الميلاد) : 24-10-1960 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 24-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
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التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: منى احمد طه خليفه National ID: 26010241400666	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Further investigations and imaging

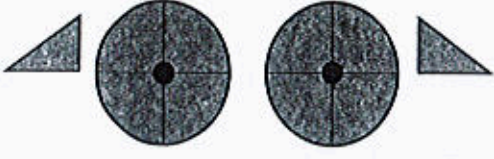
التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM 34 / 8 / 2025 G

Name	منال طحة	MRN	83164111
Age	63 yrs	ID	3568-2023
Menopausal status		Phone	01004775454
NRS SCORE		Marital status	97.1.9412.777
BMI		Family history	-ve
CO morbidity	HTN,	REASON OF MDT	change type to HT

Clinical Data

BX: IDC	R	L
ER: 8/8	PR: 6/8	HER 2: 1
Ki67: 5%		
Sonomammography:	TNM	
Mets Workup:	PTIC N1 Mo	

Recommendations: the patient had upFront BCS + ALND (1/2021)
PTIC N_{3/30} Mo
received TCX4 ECE
PORT
Start Adjuvant tam For 15 month.
T.V. US → Endometrial lining 9mm then 17cm D&C → no malignancy (3/2021)

MDT OUTCOME & PLAN

(Ad Adj AI + Radiotherapy)

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					



Patient Name: Mona Ahmed Taha Khalifa	Date : 20/09/2023
Exam : C.T CHEST	Age : 62 Year(s)
Medical ID: 3568/2023	Pat. ID: 42104

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- LEFT breast UIQ irregular speculated mass lesions the largest is measuring 2.2 X 1.8 cm.
- Right upper lung lobe posterior segment patchy consolidative opacity is noted.
- No sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions on non contrast basis
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen on noncontrast basis.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- D4 tiny sclerotic focus is noted mostly bone island.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Reported case of breast malignancy.***

Patient Name : الأستاذ / منى احمد طه خليفه :	Request No. : 13009003
Patient No. : 26010241400666	Request Date : 31/07/2025
Path No. : 2415/25	Verification Date : 04/08/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 64 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Patient on hormonal treatment.

Nature of Specimen: Dilation & Curettage, collected on 30/07/2025.

Gross Examination:

- Multiple fragmented red & glistening tan tissue fragments collectively measured 1x1 cm, totally submitted.

Microscopic Examination:

- Histopathological examination reveals portions of polypoid ectocervical tissue showing focal surface erosion. There is portion of substantia propria showing blood vessels & chronic inflammatory cells. No endometrial tissue included.
There is no evidence of malignancy in section examined.

Diagnosis:

Dilation & Curettage:

Polypoid ectocervical tissue with focal surface erosion & chronic inflammation, No malignancy.

Signature

Dr. Walaa Ibraheem Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 18/08/2025 11:23:39

Page 1 of 1

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذة / منى احمد طه خليفه
Patient No. : 26010241400666
Path No. : 3619/23
Sex : Female
Age : 62 Y
Referring Doc :

Request No. : 13006748
Request Date : 21/10/2023
Verification Date : 11/11/2023
Financial Category : نفقة الدولة
Department : اورام دار السلام - نفقة الدولة
Registered by : Dr.Marwa mahmoud

Pathology Report

• Stages : pT1c , pN1

Dr.Rana Fathy

Dr.Heba Hassan

ied by :
ba Hasan

02/2025 09:53:12

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name: Mona Ahmed Taha Khalifa	Date : 30/06/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 63 Year(s)
Medical ID: 3568/2023	Pat. ID: 42104

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:**Liver: -**

- Enlarged in size (20cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites**Urinary Bladder: -**

- Average capacity with no stones or diverticular out pouching

Uterus and adnexa: the uterus is average size, AVF, showing endometrial thickening with tiny cystic areas measuring about 17mm, no myometrial sizable lesions

Opinion:

Bright hepatomegaly likely fatty

Thickened endometrium with cystic areas? cystic endometrial hyperplasia

Marked colonic gaseous distention

Signature Dr . Nermien Ramadan

30/06/2025 11:27 AM



Patient Name : الأستاذة / منى أحمد طه خليله
Patient No. : 26010241400666
Path No. : 3619/23
Sex : Female
Age : 62 Y
Referring Doc :

Request No. : 13006749
Request Date : 21/10/2023
Verification Date : 11/11/2023
Financial Category : نفقة الدولة
Department : أورام دار السلام - Out Patient
Registered by : Dr.Marwa mahmoud

Pathology Report

• Stage : pT1c, pN1

Dr.Rana Fathy

Dr.Heba Hassan

Reviewed by :
Dr. Heba Hassan

02/2025 09:53:12

Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأسماء / م. أحمد طه علي
Patient No : 26010241400666
Path No : 3619/23
Sex : Female
Age : 62 Y
Referring Doc :

Request No : 13006749
Request Date : 21/10/2023
Verification Date : 11/11/2023
Financial Category : فئة الدولة
Department : Out Patient
Registered by : Dr. Marwa mahmoud

Pathology Report

Clinical Data : Left breast cancer
Nature of the specimen : Left BCS & SLNBlopsy & left ALN biopsy

Gross Examination:
• Procedure: Left BCS specimen measured 5x4 cm , covering skin ellipse measured 3 x0.7 cm .
• Tumor extent : A well defined greyish white firm mass measured 1.8x1.5x1 cm .
• Distance from margins: 0.1 cm away from inferior margin (least margin)
• Lymph nodes number and size: A fibrofatty tissue piece measured 4x3x1 cm enclosing 3 SLNs up to 1.5x1 cm ALND: Fibrofatty tissue 7x3.5x2 cm, entangling 28 nodes up to 1x1 cm.
• Frozen section: suspected inferior margin for DCIS & Positive SLNs (1/3) with capsular invasion

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): G II(3+2+1)
- Angiolinvasion: No
- Ductal carcinoma in situ: present
- Lymph node involvement: Positive SLNs (1/3) with capsular invasion & Positive axillary lymph nodes (2/28) with extracapsular extension
- Resection Margins: Inferior margin showed atypical crushed cells , free other margins including extra inferior margin

Diagnosis:

Left BCS & Left ALN biopsy:
Invasive duct carcinoma Grade II with DCIS
• Positive LNs (3/30) with extracapsular extension
• Free margins including extra margin

Verified by :
Dr. Heba Hasan



**GUSTAVE/
ROUSSY/**
INTERNATIONAL

NAME: mona ahmed taha

DATE: 20/4/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver:-

- Enlarged in size (19cm at Rt MCL) bright parenchymal echo pattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder:-

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen:-

Enlarged 14cm, homogenous.

Kidneys:-

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder:-

- Average capacity with no stones or diverticular out pouching
- **Uterus and adnexal:** no abnormalities could be detected

Opinion:

Bright hepatomegaly likely fatty
Mild splenomegaly
Marked colonic gaseous distention

Best regards
Dr. NERMEEN RAMADAN FEBR, MD

10/10/2023
10/10/2023
10/10/2023

Patient's Name:

MONA AHMED TAHHA

Date:

2026-04-20

Referred by:

Prof. Dr.

Hospital:

Constance Henessey International Egypt

**BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH
RESOLUTION BREAST SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left CBS

FINDINGS:

LEFT BREAST:

- Fairly dense fibroglandular densities, (ACR b), with focal parenchymal disruption seen in the UIQ associated with dystrophic calcifications as well as skin and trabecular thickening.
- No spiculations, masses, asymmetry or suspicious microcalcifications.
- Ultrasound demonstrated a well defined mixed echogenic lesion underlying the scar site, measuring 20x19mm. Colour Doppler depicted no abnormal flow.
- Clear left axilla, and no pathological infra or supra-clavicular lymph nodes.

RIGHT BREAST:

- Fairly dense fibroglandular densities, (ACR b).
- Very faint benign looking nodules are seen with no spiculations, masses, asymmetry, or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions
- Normal skin/nipple/areola complex.
- Non-specific axillary nodes.

CONCLUSION:

Left breast (BIRADS 2): Post-therapeutic sequelae with fat necrosis, for follow up
Right breast (BIRADS 2): benign findings

Best regards,
Dr. Wani Mokhtar

NAME: mona ahmed taha

DATE: 20/4/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver:-

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- Average distention and capacity with no stones or mud
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- CBD is not dilated.

Spleen:-

Enlarged 14cm, homogenous.

Kidneys:-

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder:-

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright hepatomegaly likely fatty

Mild splenomegaly

Marked colonic gaseous distention

Best regards

Dr. NERMEEN RAMADAN FEBR, MD

Patient's Name:

MONA AHMED TAHHA

Date:

2026-04-20

Referred by:

Prof. Dr.

Hospital:

Constance Ramsey International Egypt

**BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH
RESOLUTION BREAST SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left CBS

FINDINGS:

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- Non-specific axillary nodes.

CONCLUSION:

Left breast (BIRADS 2): Post-therapeutic sequelae with fat necrosis, for follow up
Right breast (BIRADS 2): benign findings

Best regards,
Dr. Wani Mokhtar